

Distinguish superficial from deep inflammation

Pain severity, vision and vessel blanching help identify the inflamed layer.

Compare the examination

Feature	Episcleritis / scleritis
Pain	Mild discomfort / severe constant deep boring pain
Vision	Usually preserved / may be reduced
Vessels	Blanch with phenylephrine / do not blanch
Appearance	Localised or diffuse injection / violaceous hue and tenderness

Use the whole pattern

Episcleritis is usually idiopathic and self-limiting; uncomplicated cases may need reassurance or lubricants. Scleritis is full-thickness inflammation and can threaten sight. Pain worsening with eye movement reflects the extraocular muscle insertions in sclera. Reduced vision, marked pain or intraocular inflammation should prompt ophthalmic assessment rather than routine reassurance. Discharge suggests an alternative or accompanying surface process, rather than being the main feature of scleritis.

Posterior disease and systemic context

Posterior scleritis can reduce vision through posterior structural involvement even without a conspicuous anterior lesion.

Think beyond the visible sclera

Posterior signs

Disc swelling, macular oedema and exudative retinal detachment can accompany posterior scleritis.

B-scan support

The T sign reflects scleral thickening and fluid in Tenon space.

Systemic association

Rheumatoid arthritis is the most common inflammatory association; ask about joints and other systemic symptoms.

Coordinate treatment

Clinical assessment guides the diagnosis, with B-scan helping in posterior disease. Rheumatological blood tests are selected to the systemic suspicion. Ophthalmology treatment may include anti-inflammatory systemic therapy, while associated disease is co-managed with rheumatology. Recurrence warrants follow-up. The reassuring course of episcleritis should not be extrapolated to deep, painful scleral inflammation with visual compromise.